Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

C. Pneumonia

D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<answer> D. Congenital heart anomaly with right-to-left shunt </answer>

<think> Reasoning process:

1. The baby was born after the 32nd gestational week, which is considered late preterm, and is at higher risk for respiratory distress syndrome (RDS).
2. The baby has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis, which are classic signs of respiratory distress.
3. The cyanosis is responding to oxygen, suggesting that it is not due to a cardiac issue, but rather a respiratory one.
4. The presence of a nasogastric tube was positioned without problems, which suggests that the baby is not experiencing significant respiratory distress that would require immediate surgical intervention.
5. RDS is a common condition in preterm infants, but it is not typically associated with cyanosis that responds to oxygen.
6. Tracheoesophageal fistula (B) would likely present with more significant respiratory distress and feeding difficulties, and would not typically respond to oxygen.
7. Pneumonia (C) could potentially cause cyanosis, but would typically be accompanied by other symptoms such as fever and respiratory failure.
8. A congenital heart anomaly with a right-to-left shunt (D) would cause cyanosis that is not responsive to oxygen, and would typically be accompanied by other symptoms such as clubbing and clubbing of the hands and feet.
9. Therefore, the cyanosis that is responding to oxygen and is not accompanied by other significant symptoms makes a congenital heart anomaly with a right-to-left shunt the most likely diagnosis.</think>